



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سعاد حسني عبد العظيم عبد الرحمن
المحافظة : القاهرة الحي / المدينة : عين شمس

العنوان :
تاريخ تسجيل الإستمارة : 06-10-2026

رقم البطاقة : 28601022104569 رقم التليفون : 01091442668

العمر (تاريخ الميلاد) : 02-01-1986 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: سعاد حسني عبد العظيم عبد الرحمن National ID: 28601022104569	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

06/11/2026

MDT FORM

Name: Soad Hossny Abdelazeem Abdelrahman	MRN: 8748
Age: 40 Y	ID: 28601022104569
Menopausal Status: PRE	Phone: 01091442668
NRS SCORE: PS 1	Marital Status: MARRIED
BMI:	Family History: -VE
CO Morbidity: IRRELEVANT	Reason Of MDT: CONTINUE HORMONAL TTT

Clinical Data:

BX: IDC GII
ER 8/8 **PR** 7/8 **HER** -VE **KI67**

R

L

Sonomammography:

Mets Workup:

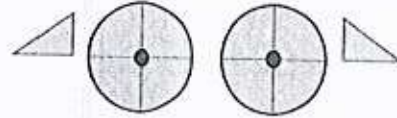
History:

CT3N1M0.
Received NACT: AC x4 cycles+ TAXOL x12 weeks
Underwent BCS in Feb 2020>>>>>Final pathology:
ypT3N0.
Completed adjuvant RT.
Started adjuvant tamoxifen in Apr 2020 and
continued until JANUARY 2024.
THEN SHIFTED TO zoladex plus letrozole since 3/

Recommendations:

MDT Outcome & Plan

To continue Femara + Zoladex for 1-2 years (total 7-8
years of ET)



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ندي حسن					





العينة

Pathology Report

Name: سعاد حمدي عبد العظيم

Age: 34 years.

Clinical data: Right breast mass, post chemotherapy.

Specimen type: Right quadrantectomy & axillary clearance.

Date: 12/2/2020

* ab. Number: 203

Hospital Number: 2169/2019

Gross Description:

A sector of mammary tissue; 12x7x5.5 cm, with partial skin covering; 10x3 cm and with attached axillary tissue; 7.5x6 cm, received unblock.

Serial cuts revealed an ill-defined firm greyish white firm greyish white fibrotic mass; 9x3.5x2.5 cm, located 2cm deep to skin and located (1, 2, 2, 2, & 2) cm from (lateral, medial, superior, inferior & deep) margins respectively.

Axillary dissection revealed 16 nodules, up to 3cm in diameter.

Microscopic Examination:

Histopathological examination of sections prepared from the quadrantectomy specimen received reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. mucoid activity is also detected. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. No intraduct component detected. No lymphovascular emboli and perineural invasion. Pathological partial response to therapy, Tumor necrosis index 35%. Free skin and resection margins. Free dissected lymph nodes 0/14 with reactive changes.

Diagnosis:

Right breast mass, post chemotherapy, quadrantectomy & axillary clearance;

- Invasive duct carcinoma with mucoid activity.
- Tumor necrosis index 35%.
- Dystrophic calcification.
- No DCIS.
- No perineural invasion.
- No lymphovascular emboli.
- Free skin. Free surgical margins.
- Lymph nodes 0/16.
- Pathological partial response to therapy.
- Tumor stage ypT3 ypN0/

Prof. Dr. Lubna Omar

Prof. Dr. Essam Ayad

[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Abdalla M. Khalil

Professor of Pathology
Faculty of Medicine
Kasr El-Aini

دكتور
عبدالله محمود خليل
أستاذ الباثولوجي - كلية طب قصر العيني
مستشار الباثولوجيا بمعهد السرطان القومي
عضو أكاديمية الباثولوجيا الدولية

Patient Name: **سعاد حسنى عبدالعظيم**

Referred by Prof. Dr.: **M. RASLAN**

Nature of the Specimen: **TRU-CUT BIOPSY**

Clinical Diagnosis: **BREAST MASS**

Receiving Date: **Jun. / 2019**

Code Number: **E 72**

PATHOLOGY REPORT

GROSS PATHOLOGY:

Multiple gray white and yellow tissue cores of total length 7.5 cm, totally submitted.

HISTOLOGY:

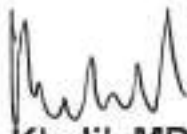
Sections examined revealed a core of breast tissue showed infiltrating nests and cords of malignant ductal cells displaying with focal lobular cells in files moderate cellular anaplasia with pleomorphic hyperchromatic nuclei in desmoplastic stroma, with focal mucinous areas and focal perineural invasion.

DIAGNOSIS:

**CORE BIOPSY: INVASIVE TERMINAL DUCT AND
LOBULAR CARCINOMA GRADE II.**

P.O.:

- 1- The results of ER, PR and Her-2 are enclosed in a separate report
- 2- Slide enclosed.


A.M. Khalil, MD

PATIENT NAME: Soad hosny Abd elazem Abd el rahman	PATIENT ID #: 2019089352
GENDER: Female	ORDER #: 485547
AGE: 33 y	DATE OF SERVICE: 27-Jun-2019 11:02 AM

Report: - all studies

**CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY,
AND COMPLEMENTARY ULTRASOUND**

INDICATION:

Right pathologically proven malignant mass for assessment.

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO view.
- Complementary ultrasound

REPORT:

- **Contrast Enhanced Spectral Mammography** showed right UOQ diffuse hetregensouly enhancing area with surrounding satellites, such area seen reaching to the underlying muscle with an overall involvement area measuring **7.3 x 10.8cm**; it shows intra-ductal extension by the complementary US extending 2cm from the nipple.
- By conventional mammogram the mass showing overlying concomitant microcalcification.
- Left breast shows mild back ground parenchymal enhancement with left UOQ microlobulated hetregensouly enhancing lesion, by complementary US it is consistent to an oval shaped well defined isoechoic lesion measuring 0.8 x 1.6cm.
- Right pathologically enlarged axillary lymph nodes.

OPINION:

- Right UOQ (9-11 O'clock) pathologically proven malignant process with extensions as above described (**BIRADS 6**) along with infiltrated ipsilateral axillary lymph nodes.
- Left UOQ probably benign looking lesion (**BIRADS 3**), though harboring benign morphological sonographic criteria, yet pattern of enhancement warrants further biopsy confirmation owing to the presence of contralateral malignancy.

Recommendation - all studies

Referral to clinic

Name: SOAD HOSNY
Date: 1 July 2019

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

Average size (about 15 cm at the right MCL), displaying homogenous echopattern with smooth surface.

- No definite focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention with thickened wall contains two stones 6 and 15 mm
- CBD is not dilated.

Spleen: - average size, homogenous echopattern.

Kidneys: -

- Are of normal size, well differentiated & with no gross stones, or space occupying lesion could be seen.

-Para-aortic and pancreatic area: no gross lesion

-No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching
- Uterus and adenexa; no abnormalities could be detected(IUD is seen in place)

Opinion:

Chronic calcular cholecystitis

Best regards
Dr. nermeen Ramadan.MD

Dr.
Abdalla M. Khalil

Professor of Pathology
Faculty of Medicine
Kasr El-Aini

دكتور
عبدالله محمود خليل
استاذ الباثولوجي - كلية طب قصر العيني
مستشار الباثولوجيا بمعهد السرطان القومي
عضو أكاديمية الباثولوجيا الدولية

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Recommendation - all studies

Referral to clinic



Capital Scan

NAME: SOAD HOSNY ABDELAZEEM
DATE: 16/06/19
REF BY PROF DR: ABLA ALI ELSAYED

BILATERAL BREAST SONOMAMOGRAPHY REVEALED:

- A large irregular opacity seen along the right upper outer quadrant proved by complementary ultrasound to be a large fixed hypoechoic irregular solid lesion of about +/- 41X36 mm seen at 2 o'clock with few satellites around of about 18X11 and 12 X13 mm with associated enlarged hypoechoic infiltrated right axillary lymph nodes with lost hilae also noticed retracted right nipple.
- The left breast shows a +/- 15X6 mm well defined hypoechoic benign looking mass.
- Both breasts show mixed glandular and fatty elements (ACR II) with overlapping opacities.
- No duct ectasia.
- No characteristic calcifications.
- Normal skin thickness with normal left nipple.
- Few left enlarged axillary lymph nodes with preserved fatty hilae.

Conclusions:

- **FIXED RIGHT UPPER OUTER QUADRANT MALIGNANT LOOKING BREAST LESION WITH SATELLITES AND INFILTRATED RIGHT AXILLARY LYMPH NODES (BIRADS 4).**
- **LEFT UPPER OUTER QUADRANT FIBROADENOMA.**

AMR MAHMOUD, MD

Dr.
Abdalla M. Khalil

Professor of Pathology
Faculty of Medicine
Kasr El-Aini

دكتور
عبدالله محمود خليل

استاذ الباثولوجي - كلية طب قصر العيني
مستشار الباثولوجيا بمعهد السرطان القومي
عضو أكاديمية الباثولوجيا الدولية

Patient Name: **سعاد حسنى عبد العظيم**
Referred by Prof. Dr.: **M. RASLAN**
Nature of the Specimen: **ER, PrR and Her-2.**
Clinical Diagnosis: **Car. Br.**
Receiving Date: **June / 2019** Code Number: **E 72**

IMMUNOHISTOCHEMICAL REPORT

Sections were treated with monoclonal antibodies for:
ER, PrR and Her-2.

RESULTS:

Tumour cells showed:

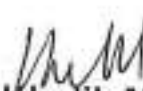
- 1- Strong positive nuclear reaction for ER (score 8/8) (95% positive cells).**
- 2- Strong positive nuclear reaction for PR (score 7/8) (85% positive cells).**
- 3- Negative over expression for Her-2 (score 0).**

CONCLUSION:

Morphology and immune profile of terminal duct carcinoma strongly positive for ER (score 8/8), strongly positive for PR (score 7/8) and negative over expression for Her-2 (score 0).

P.O.:

3 slides enclosed.


A.M. Khalil, MD

Dar El-Salam hospital
Radiology Department
قسم الأشعة

Name; soad hosny

Date : 21/02/22

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows homogenous texture with no focal lesion
- The endometrial line is 6mm. IUCD is seen in place

○ **Ovaries: -**

- Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:

Unremarkable study

Best regards

Dr / NERMEEN RAMADAN, MD



Patient Name: Souad Hosny Abdel Azeem Abd El Rahman	Date : 15/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 2169/2019	Pat. ID: 14049

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS in 1/2020

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Left breast UOQ middle third partially obscured equal density oval mass.
- Complementary ultrasound showed right breast OQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Left breast circumscribed oval shaped isoechoic mass, seen at 2 to 3 o'clock, measured 0.6x1.2cm, of stationary course since 2019
- normal skin thickness and nipple-areola complex.
- Nonspecific axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions
- Left breast 2o'clock benign looking lesion of stationary course
- (BIRADS 2)

Signature Dr. Nehal Hamdy

15/10/2023 3:55 PM



Patient Name: Souad Hosny Abdel Azeem Abd El Rahman	Date : 08/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 2169/2019	Pat. ID: 14049

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS in

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- stable left breast solid mass lesion measured 11x5mm, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- left breast UOQ stable small adenoma (BIRADS 2)

Yours truly

Signature Dr basma mohmaed

08/12/2024 12:30 PM



Patient Name: Souad Hosny Abdel Azeem Abd El Rahman	Date : 08/12/2024
Exam : Item 1	Age : 39 Year(s)
Medical ID: Kholoud Zaher^Head	14049

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1 WIs
- Post contrast series

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal.
- Bilaterally ectatic optic nerves sheaths with partial empty sella, enlarged Meckel's caves.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

- No definite enhancing intra cranial masses.
- Some features are suggestive of increased intracranial tension, for clinical correlation and evaluation by MRV if clinically indicated.

Dr. Sherif Sameh, M.D.



Name : Soaad Hosny Abdelazim

Date : 21-02-2022

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

A known case of breast cancer.

Technique of examination:

- Sagittal T1 & T2 WIs.
- Axial T2 WIs.
- No IV contrast was given.

MRI Findings:

- Preserved lumbar lordotic curvature.
- Normal diameters of the bony lumbar spinal canal.
- No evidence of significant disc bulge or protrusion.
- Normal MRI appearance of the conus medullaris and cauda equina nerve roots.
- Normal marrow signal of the imaged vertebrae.
- No paraspinal signal abnormalities.

Opinion:

- **Normal MRI study of the lumbo-sacral spine.**

Dr. Reham Anwar, MSc, FEBR

Dr. Amr Farouk, M.D.